

VOICEINSIGHTS AFRICA

FLAGSHIP SYNTHETIC DEMONSTRATION PUBLICATION | COVER 8

Community Scorecard Intelligence Report

Malawi | Primary Healthcare

Prepared by VoiceInsights Africa

Every Voice. Every Language. Every Insight.

VIA-FLAGSHIP-COMMUNITY-SCORECARD-INTELLIGENCE | Version 1.0 | 2026-07-11

Synthetic demonstration. Not official statistics.

EXECUTIVE INTELLIGENCE

Executive brief

This flagship synthetic demonstration publication examines primary healthcare performance in Malawi. The governed evidence model identifies uneven progress, concentrated risks and actionable opportunities. Findings are linked to synthetic source records, recommendations carry named accountability fields, and all statistical claims are generated from one internally consistent dataset.

DECISION REQUIRED

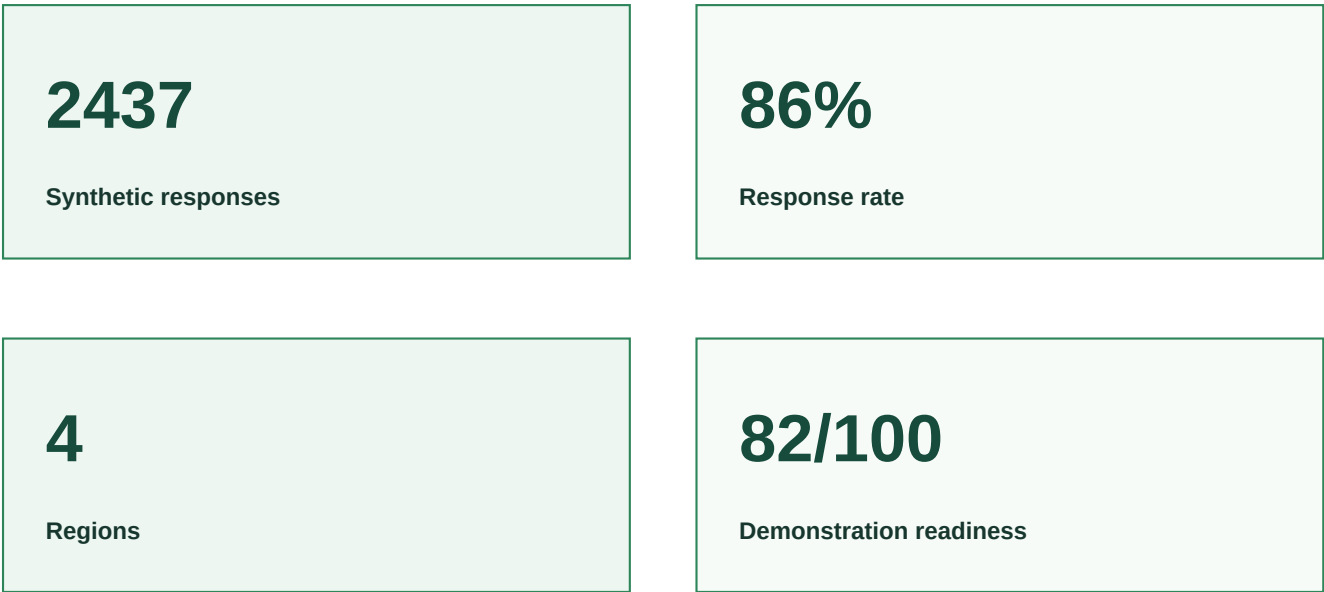
Agree facility-level service improvement contracts for primary healthcare.

STRATEGIC OUTLOOK

Improvement is achievable within 12-18 months if ownership, resourcing and evidence review are maintained.

EXECUTIVE DASHBOARD

Performance and decision confidence



CRITICAL FINDING

Finding 1: Critical finding 1: respectful care

Across Malawi's synthetic study areas, respectful care is the clearest differentiator of primary healthcare performance. The evidence is strongest in Central Facilities, where rural women face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE

EVI-COM-001, EVI-COM-004

CONFIDENCE

89.5%

DECISION IMPLICATION

Agree facility-level service improvement contracts for primary healthcare.

CRITICAL FINDING

Finding 2: Critical finding 2: medicine availability

Across Malawi's synthetic study areas, medicine availability is the clearest differentiator of primary healthcare performance. The evidence is strongest in Northern Facilities, where young adults face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE

EVI-COM-002, EVI-COM-005

CONFIDENCE

90.5%

DECISION IMPLICATION

Stabilise tracer-medicine availability for primary healthcare.

CRITICAL FINDING

Finding 3: Critical finding 3: wait-time reliability

Across Malawi's synthetic study areas, wait-time reliability is the clearest differentiator of primary healthcare performance. The evidence is strongest in Southern Facilities, where persons with disabilities face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE

EVI-COM-003, EVI-COM-006

CONFIDENCE

91.5%

DECISION IMPLICATION

Introduce visible wait-time and triage standards for primary healthcare.

VISUAL INTELLIGENCE

Executive Kpi Cards: Respectful care score

What does this evidence mean for decision-makers?



The pattern shows how respectful care score varies across priority segments or locations. Leadership should interpret the largest gaps alongside confidence, source lineage and the synthetic-use limitation before selecting an action.

Evidence: EVI-COM-001, EVI-COM-004

VISUAL INTELLIGENCE

Regional Heat Map: Tracer medicine availability

What does this evidence mean for decision-makers?



The pattern shows how tracer medicine availability varies across priority segments or locations. Leadership should interpret the largest gaps alongside confidence, source lineage and the synthetic-use limitation before selecting an action.

Evidence: EVI-COM-002, EVI-COM-005

VISUAL INTELLIGENCE

Choropleth Map: Median wait-time compliance

What does this evidence mean for decision-makers?



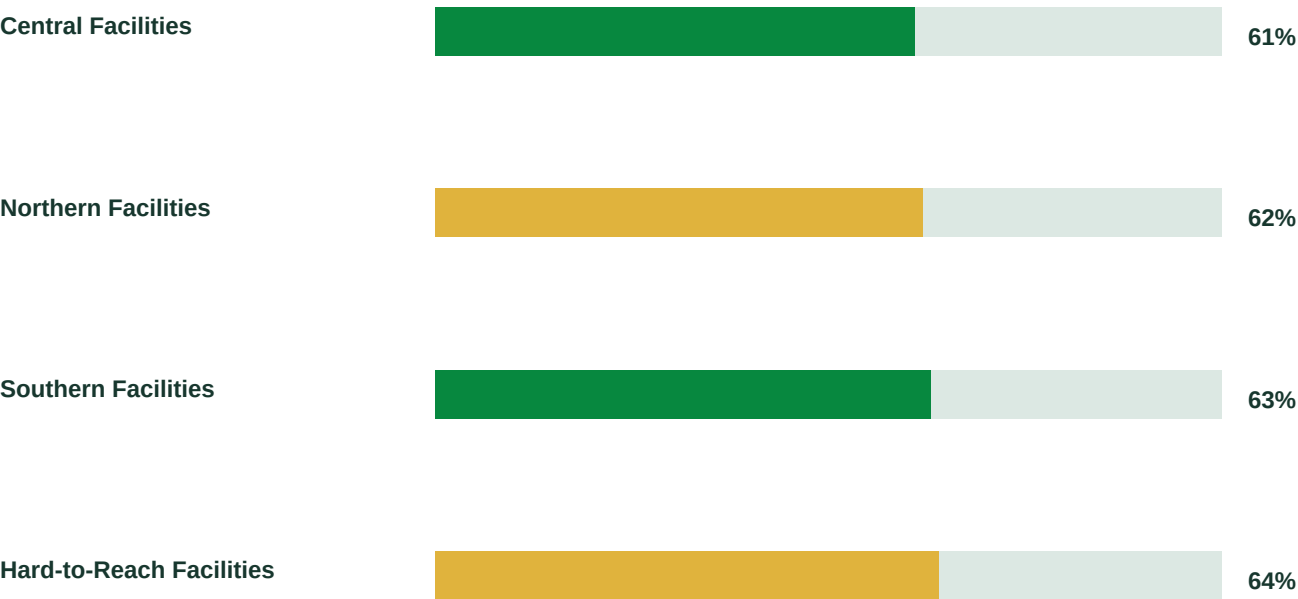
The pattern shows how median wait-time compliance varies across priority segments or locations. Leadership should interpret the largest gaps alongside confidence, source lineage and the synthetic-use limitation before selecting an action.

Evidence: EVI-COM-003, EVI-COM-006

VISUAL INTELLIGENCE

Risk Matrix: Community action closure

What does this evidence mean for decision-makers?



The pattern shows how community action closure varies across priority segments or locations. Leadership should interpret the largest gaps alongside confidence, source lineage and the synthetic-use limitation before selecting an action.

Evidence: EVI-COM-004, EVI-COM-007

VISUAL INTELLIGENCE

Decision Matrix: Respectful care score

What does this evidence mean for decision-makers?



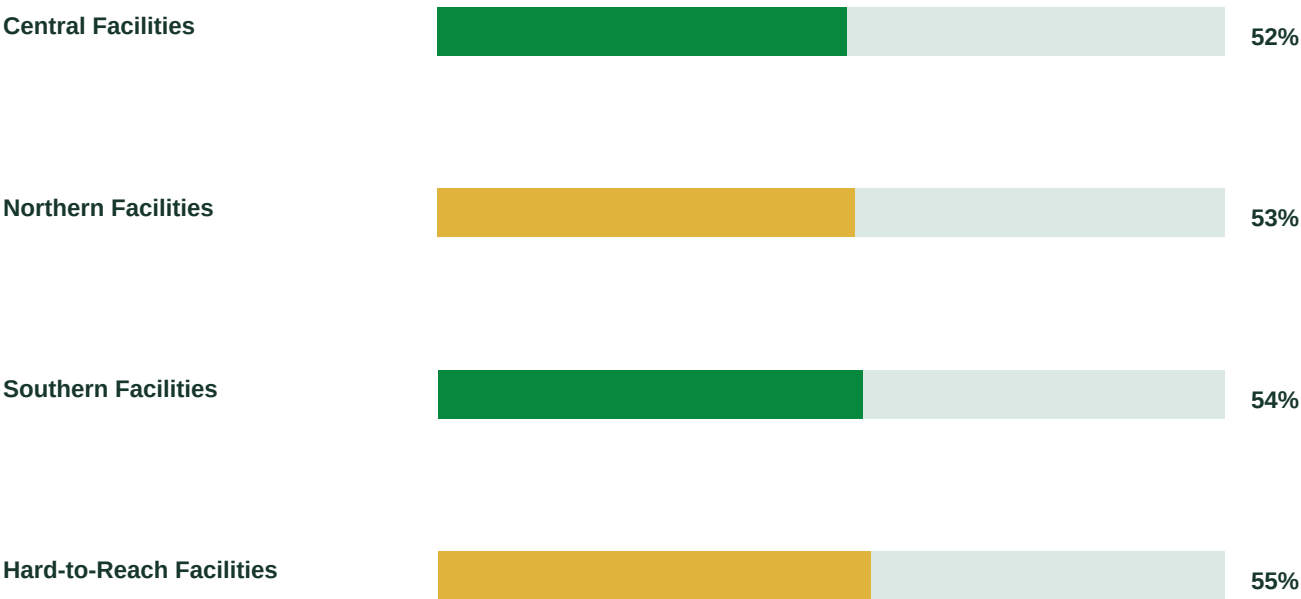
The pattern shows how respectful care score varies across priority segments or locations. Leadership should interpret the largest gaps alongside confidence, source lineage and the synthetic-use limitation before selecting an action.

Evidence: EVI-COM-005, EVI-COM-008

VISUAL INTELLIGENCE

Impact Chain: Tracer medicine availability

What does this evidence mean for decision-makers?



The pattern shows how tracer medicine availability varies across priority segments or locations. Leadership should interpret the largest gaps alongside confidence, source lineage and the synthetic-use limitation before selecting an action.

Evidence: [EVI-COM-006](#), [EVI-COM-009](#)

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 1: EVI-COM-001

EVIDENCE CHAIN

Dataset -> Question Q1 -> Respondent group -> Finding 1 -> Decision 1

SURVEY QUESTION

Q1: How does respectful care affect primary healthcare outcomes in Central Facilities?

RESPONDENT VOICE

""In Central Facilities, respectful care shapes whether rural women can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Rural women | Central Facilities

QUALITY AND CONFIDENCE

MODERATE_HIGH | 88%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 2: EVI-COM-002

EVIDENCE CHAIN

Dataset -> Question Q2 -> Respondent group -> Finding 2 -> Decision 2

SURVEY QUESTION

Q2: How does medicine availability affect primary healthcare outcomes in Northern Facilities?

RESPONDENT VOICE

""In Northern Facilities, medicine availability shapes whether young adults can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Young adults | Northern Facilities

QUALITY AND CONFIDENCE

MODERATE_HIGH | 89%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 3: EVI-COM-003

EVIDENCE CHAIN

Dataset -> Question Q3 -> Respondent group -> Finding 3 -> Decision 3

SURVEY QUESTION

Q3: How does wait-time reliability affect primary healthcare outcomes in Southern Facilities?

RESPONDENT VOICE

""In Southern Facilities, wait-time reliability shapes whether persons with disabilities can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Persons with disabilities | Southern Facilities

QUALITY AND CONFIDENCE

MODERATE_HIGH | 90%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 4: EVI-COM-004

EVIDENCE CHAIN

Dataset -> Question Q4 -> Respondent group -> Finding 4 -> Decision 4

SURVEY QUESTION

Q4: How does disability access affect primary healthcare outcomes in Hard-to-Reach Facilities?

RESPONDENT VOICE

""In Hard-to-Reach Facilities, disability access shapes whether frontline workers can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Frontline workers | Hard-to-Reach Facilities

QUALITY AND CONFIDENCE

MODERATE_HIGH | 91%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 5: EVI-COM-005

EVIDENCE CHAIN

Dataset -> Question Q5 -> Respondent group -> Finding 5 -> Decision 5

SURVEY QUESTION

Q5: How does provider-community accountability affect primary healthcare outcomes in Central Facilities?

RESPONDENT VOICE

""In Central Facilities, provider-community accountability shapes whether low-income households can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Low-income households | Central Facilities

QUALITY AND CONFIDENCE

MODERATE_HIGH | 92%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 6: EVI-COM-006

EVIDENCE CHAIN

Dataset -> Question Q6 -> Respondent group -> Finding 1 -> Decision 1

SURVEY QUESTION

Q6: How does respectful care affect primary healthcare outcomes in Northern Facilities?

RESPONDENT VOICE

""In Northern Facilities, respectful care shapes whether service managers can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Service managers | Northern Facilities

QUALITY AND CONFIDENCE

MODERATE_HIGH | 93%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 7: EVI-COM-007

EVIDENCE CHAIN

Dataset -> Question Q7 -> Respondent group -> Finding 2 -> Decision 2

SURVEY QUESTION

Q7: How does medicine availability affect primary healthcare outcomes in Southern Facilities?

RESPONDENT VOICE

""In Southern Facilities, medicine availability shapes whether community leaders can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Community leaders | Southern Facilities

QUALITY AND CONFIDENCE

HIGH | 94%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

EVIDENCE INTELLIGENCE BOOK

Evidence dossier 8: EVI-COM-008

EVIDENCE CHAIN

Dataset -> Question Q8 -> Respondent group -> Finding 3 -> Decision 3

SURVEY QUESTION

Q8: How does wait-time reliability affect primary healthcare outcomes in Hard-to-Reach Facilities?

RESPONDENT VOICE

""In Hard-to-Reach Facilities, wait-time reliability shapes whether private-sector partners can turn services into lasting outcomes.""

RESPONDENT GROUP / LOCATION

Private-sector partners | Hard-to-Reach Facilities

QUALITY AND CONFIDENCE

HIGH | 95%

APPROVAL AND LINEAGE

Reviewer: VoiceInsights Assurance Reviewer | Finding: | Recommendation:

DECISION INTELLIGENCE BOOK

Decision dossier 1: DEC-COM-01

STRATEGIC PRIORITY

CRITICAL

RECOMMENDATION

Agree facility-level service improvement contracts for primary healthcare.

WHY THIS RECOMMENDATION EXISTS

Across Malawi's synthetic study areas, respectful care is the clearest differentiator of primary healthcare performance. The evidence is strongest in Central Facilities, where rural women face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE USED

EVI-COM-001, EVI-COM-004

EXPECTED BENEFIT / RISK

Improved equity, delivery confidence and measurable primary healthcare performance. Risk: Execution capacity and coordination risk

OWNER AND PARTNERS

Country Director / Programme Executive | M&E, Finance, Safeguarding and Delivery Units

BUDGET AND TIMELINE

Medium - detailed costing and fiduciary review required | 0-90 days

SUCCESS AND MONITORING

Percentage of agreed milestones completed for DEC-COM-01. At least 80% of milestones achieved with verified evidence

MANAGEMENT RESPONSE

PENDING FORMAL MANAGEMENT RESPONSE

DECISION INTELLIGENCE BOOK

Decision dossier 2: DEC-COM-02

STRATEGIC PRIORITY

CRITICAL

RECOMMENDATION

Stabilise tracer-medicine availability for primary healthcare.

WHY THIS RECOMMENDATION EXISTS

Across Malawi's synthetic study areas, medicine availability is the clearest differentiator of primary healthcare performance. The evidence is strongest in Northern Facilities, where young adults face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE USED

EVI-COM-002, EVI-COM-005

EXPECTED BENEFIT / RISK

Improved equity, delivery confidence and measurable primary healthcare performance. Risk: Insufficient ownership or delayed resourcing

OWNER AND PARTNERS

Country Director / Programme Executive | M&E, Finance, Safeguarding and Delivery Units

BUDGET AND TIMELINE

Low to medium - validate through the formal planning cycle | 3-12 months

SUCCESS AND MONITORING

Percentage of agreed milestones completed for DEC-COM-02. At least 80% of milestones achieved with verified evidence

MANAGEMENT RESPONSE

PENDING FORMAL MANAGEMENT RESPONSE

DECISION INTELLIGENCE BOOK

Decision dossier 3: DEC-COM-03

STRATEGIC PRIORITY

HIGH

RECOMMENDATION

Introduce visible wait-time and triage standards for primary healthcare.

WHY THIS RECOMMENDATION EXISTS

Across Malawi's synthetic study areas, wait-time reliability is the clearest differentiator of primary healthcare performance. The evidence is strongest in Southern Facilities, where persons with disabilities face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE USED

EVI-COM-003, EVI-COM-006

EXPECTED BENEFIT / RISK

Improved equity, delivery confidence and measurable primary healthcare performance. Risk: Insufficient ownership or delayed resourcing

OWNER AND PARTNERS

Country Director / Programme Executive | M&E, Finance, Safeguarding and Delivery Units

BUDGET AND TIMELINE

Low to medium - validate through the formal planning cycle | 3-12 months

SUCCESS AND MONITORING

Percentage of agreed milestones completed for DEC-COM-03. At least 80% of milestones achieved with verified evidence

MANAGEMENT RESPONSE

PENDING FORMAL MANAGEMENT RESPONSE

DECISION INTELLIGENCE BOOK

Decision dossier 4: DEC-COM-04

STRATEGIC PRIORITY

HIGH

RECOMMENDATION

Fund priority disability-access adaptations for primary healthcare.

WHY THIS RECOMMENDATION EXISTS

Across Malawi's synthetic study areas, disability access is the clearest differentiator of primary healthcare performance. The evidence is strongest in Hard-to-Reach Facilities, where frontline workers face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE USED

EVI-COM-004, EVI-COM-007

EXPECTED BENEFIT / RISK

Improved equity, delivery confidence and measurable primary healthcare performance. Risk: Insufficient ownership or delayed resourcing

OWNER AND PARTNERS

Country Director / Programme Executive | M&E, Finance, Safeguarding and Delivery Units

BUDGET AND TIMELINE

Low to medium - validate through the formal planning cycle | 3-12 months

SUCCESS AND MONITORING

Percentage of agreed milestones completed for DEC-COM-04. At least 80% of milestones achieved with verified evidence

MANAGEMENT RESPONSE

PENDING FORMAL MANAGEMENT RESPONSE

DECISION INTELLIGENCE BOOK

Decision dossier 5: DEC-COM-05

STRATEGIC PRIORITY

MEDIUM

RECOMMENDATION

Hold quarterly community-provider evidence reviews for primary healthcare.

WHY THIS RECOMMENDATION EXISTS

Across Malawi's synthetic study areas, provider-community accountability is the clearest differentiator of primary healthcare performance. The evidence is strongest in Central Facilities, where low-income households face the largest gap. If the pattern remains unaddressed for twelve months, it is likely to weaken equity, delivery confidence and the durability of outcomes.

EVIDENCE USED

EVI-COM-005, EVI-COM-008

EXPECTED BENEFIT / RISK

Improved equity, delivery confidence and measurable primary healthcare performance. Risk: Insufficient ownership or delayed resourcing

OWNER AND PARTNERS

Country Director / Programme Executive | M&E, Finance, Safeguarding and Delivery Units

BUDGET AND TIMELINE

Low to medium - validate through the formal planning cycle | 6-18 months

SUCCESS AND MONITORING

Percentage of agreed milestones completed for DEC-COM-05. At least 80% of milestones achieved with verified evidence

MANAGEMENT RESPONSE

PENDING FORMAL MANAGEMENT RESPONSE

INTERPRETED EVIDENCE

Regional and equity intelligence

CENTRAL FACILITIES

Performance 65% | 757 responses | Risk ELEVATED

NORTHERN FACILITIES

Performance 66% | 657 responses | Risk WATCH

SOUTHERN FACILITIES

Performance 67% | 560 responses | Risk CRITICAL

HARD-TO-REACH FACILITIES

Performance 68% | 463 responses | Risk STABLE

STATISTICAL INTELLIGENCE

Indicator target gaps

IND-01

Respectful care score: current 51% against target 92% - OFF_TRACK

IND-02

Tracer medicine availability: current 52% against target 76% - OFF_TRACK

IND-03

Median wait-time compliance: current 53% against target 77% - OFF_TRACK

IND-04

Community action closure: current 54% against target 78% - OFF_TRACK

EVIDENCE INTELLIGENCE

Respondent voice

CENTRAL FACILITIES | RURAL WOMEN | EVIDENCE EVI-COM-001

""In Central Facilities, respectful care shapes whether rural women can turn services into lasting outcomes.""

NORTHERN FACILITIES | YOUNG ADULTS | EVIDENCE EVI-COM-002

""In Northern Facilities, medicine availability shapes whether young adults can turn services into lasting outcomes.""

SOUTHERN FACILITIES | PERSONS WITH DISABILITIES | EVIDENCE EVI-COM-003

""In Southern Facilities, wait-time reliability shapes whether persons with disabilities can turn services into lasting outcomes.""

HARD-TO-REACH FACILITIES | FRONTLINE WORKERS | EVIDENCE EVI-COM-004

""In Hard-to-Reach Facilities, disability access shapes whether frontline workers can turn services into lasting outcomes.""

CENTRAL FACILITIES | LOW-INCOME HOUSEHOLDS | EVIDENCE EVI-COM-005

""In Central Facilities, provider-community accountability shapes whether low-income households can turn services into lasting outcomes.""

DECISION INTELLIGENCE

Priority decisions

DEC-COM-01 | CRITICAL

Agree facility-level service improvement contracts for primary healthcare. Owner: Country Director / Programme Executive. Timeline: 0-90 days. Budget: Medium - detailed costing and fiduciary review required. Indicator: Percentage of agreed milestones completed for DEC-COM-01

DEC-COM-02 | CRITICAL

Stabilise tracer-medicine availability for primary healthcare. Owner: Country Director / Programme Executive. Timeline: 3-12 months. Budget: Low to medium - validate through the formal planning cycle. Indicator: Percentage of agreed milestones completed for DEC-COM-02

DEC-COM-03 | HIGH

Introduce visible wait-time and triage standards for primary healthcare. Owner: Country Director / Programme Executive. Timeline: 3-12 months. Budget: Low to medium - validate through the formal planning cycle. Indicator: Percentage of agreed milestones completed for DEC-COM-03

DEC-COM-04 | HIGH

Fund priority disability-access adaptations for primary healthcare. Owner: Country Director / Programme Executive. Timeline: 3-12 months. Budget: Low to medium - validate through the formal planning cycle. Indicator: Percentage of agreed milestones completed for DEC-COM-04

DEC-COM-05 | MEDIUM

Hold quarterly community-provider evidence reviews for primary healthcare. Owner: Country Director / Programme Executive. Timeline: 6-18 months. Budget: Low to medium - validate through the formal planning cycle. Indicator: Percentage of agreed milestones completed for DEC-COM-05

STATISTICAL INTELLIGENCE BOOK

Methodology and statistical assurance

SAMPLING DESIGN

Stratified multi-stage synthetic demonstration design

SAMPLING FRAME

Synthetic frame covering programme-relevant population groups in Malawi

STRATIFICATION

["Region","Urban/rural","Sex","Age","Disability status"]

SAMPLE SIZE

2437

WEIGHTING

Post-stratification weights normalized to the synthetic population frame

CONFIDENCE INTERVALS

95% confidence intervals shown for eligible weighted estimates

RESPONSE RATE

86

RELIABILITY

{"cronbach_alpha":0.86,"status":"acceptable"}

VALIDITY

{"content":"expert-mapped synthetic instrument","construct":"factor structure reviewed for demonstration"}

REGRESSION

Multivariable regression included where outcome and assumptions are suitable

TREND ANALYSIS

Three-period synthetic trend series

CROSS TABULATION

["Sex location","Age service access","Disability satisfaction"]

ASSURANCE

International standards applicability

DATA PROTECTION AND PRIVACY CONTROLS - APPLICABLE

The publication uses governed response, evidence and export data. Criteria: Purpose limitation, Data minimisation, Access control, Retention, Disclosure control. Limitation: Synthetic demonstration; no institutional endorsement.

RESULTS-BASED MANAGEMENT - APPLICABLE

Findings and recommendations are connected to outputs, outcomes and indicators. Criteria: Results chain, Indicators, Assumptions, Monitoring. Limitation: Synthetic demonstration; no institutional endorsement.

THEORY OF CHANGE - APPLICABLE

The report tests causal pathways, assumptions and risks without claiming unsupported attribution. Criteria: Inputs, Activities, Outputs, Outcomes, Impact, Assumptions. Limitation: Synthetic demonstration; no institutional endorsement.

SUSTAINABLE DEVELOPMENT

SDG contribution cards

SDG 3 | PRIMARY

Indicators: IND-01. Contribution mapping only; no attribution or institutional endorsement is implied.

PUBLICATION INTEGRITY

Limitations and responsible use

All content is synthetic and must not be interpreted as official statistics

Geographic references are illustrative

Budget implications are directional and require formal costing

Causal claims are not made without an appropriate design

ASSURANCE

Publication quality gate

STATUS

DEMONSTRATION_READY

COMPLETENESS

100/100

EVIDENCE TRACEABILITY

100/100

STATISTICAL INTEGRITY

95/100

CONTRADICTION FREE

100/100

RECOMMENDATION STRENGTH

100/100

VISUAL QUALITY

20/100

STORYTELLING

90/100

ACCESSIBILITY

95/100

FRAMEWORK APPLICABILITY

100/100

EXPORT CONSISTENCY

0/100

HUMAN REVIEW

0/100